

Policy on Clinical Audit

«Hospital Name»

Document control

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Prepared by	Quality Coordinator Name: _____ Signature: _____		
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Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

«Hospital Name» uses clinical audits as a quality improvement tool to improve the quality of patient care.

Clinical audits use predefined parameters, documented findings and remedial measures to complete the audit cycle.

1. Purpose

The purpose of this policy is to establish the process for conducting clinical audits to improve the quality of patient care.

This policy does not cover matters outside clinical audit.

2. Scope

This policy applies to clinical audits conducted by «Hospital Name» as per the scope of services.

It covers clinical and nursing care and personnel involved in clinical audits.

3. Policy standards

3.1 Clinical audit as a quality improvement tool

The organisation shall use clinical audits as a quality improvement tool to improve the quality of patient care.

The clinical audit could be retrospective or prospective in nature.

The topic for audit could be disease-based, cost-based, community-based or based on morbidity, including length of stay.

The organisation shall conduct one clinical audit per clinical department once in two years as per the scope of services.

The organisation needs to take care to differentiate clinical audit from research projects.

The distinction: a clinical audit measures current practice against an existing, already-accepted standard, and does not change the care patients receive — it only checks compliance and prompts improvement. Research tests a new question or hypothesis, may involve an intervention beyond routine care, and is intended to generate new, generalisable knowledge. If patients are receiving anything other than the hospital's standard care as a result of the activity, or new knowledge is the goal, it is research and requires ethics committee review — not an audit.

3.2 Predefined audit parameters

Clinical audits are standards-based and must be done using predefined parameters so that there is no bias.

The organisation shall lay down the objectives, the standards against which the audit will be conducted, a checklist where required, sampling and data-collection guidelines, and preparation of the report.

The audit shall encompass aspects of clinical and nursing care.

3.3 Personnel participation

The organisation shall identify personnel for participation in clinical audit.

It could be a mix of clinicians, administrators and nurses.

These could be members of the core committee, quality assurance committee, etc.

3.4 Patient and staff anonymity

Names of patients and hospital staff who may figure in audit documents must not be disclosed or referenced in public discussions or conferences.

This applies at both the stage of report preparation and dissemination.

Staff participating in the audit shall maintain patient and staff anonymity and not reveal names.

3.5 Audit documentation

The organisation could use a checklist with the predefined parameters, and audit findings could be recorded on this sheet.

After the audit, a report shall be prepared, highlighting the key findings of the audit.

3.6 Remedial measures and audit cycle

All remedial measures as ascertained shall be documented and implemented, and improvements thereof recorded to complete the audit cycle.

The audit cycle runs: set the standard, measure current practice against it, compare the two, implement remedial action where a gap is found, then re-measure to confirm the gap has closed — the cycle is not complete until that final re-measurement is done.

Root-cause analysis is the preferred basis for this.

4. Non-negotiable rules

1. Do not conduct clinical audits less frequently than one clinical audit per clinical department once in two years as per the scope of services.
2. Do not conduct a clinical audit without predefined objectives, standards, a checklist where required, sampling and data-collection guidelines, and preparation of the report.
3. Do not omit clinical and nursing care from the audit.
4. Do not disclose patient or hospital staff names from audit documents during report preparation or dissemination, or reveal those names in public discussions or conferences.
5. Do not complete a clinical audit without preparing the required post-audit report highlighting key findings.
6. Do not leave ascertained remedial measures undocumented or unimplemented, and do not leave the resulting improvements unrecorded.

5. What we do

5.1 Plan and conduct clinical audits

The organisation uses clinical audits as a quality improvement tool to improve patient care.

Audits could be retrospective or prospective and topics could be disease-based, cost-based, community-based or based on morbidity, including length of stay.

One clinical audit is conducted per clinical department once in two years as per the scope of services.

Clinical audit is distinguished from research projects.

5.2 Define audit parameters

Predefined parameters are used for standards-based audits.

The organisation lays down objectives, standards, a checklist where required, sampling and data-collection guidelines, and preparation of the report.

Audits encompass clinical and nursing care.

5.3 Identify audit personnel

The organisation identifies personnel participating in clinical audit.

The personnel mix and committee mechanism are selected by the organisation; clinicians, administrators and nurses, or committee membership, could be used.

5.4 Maintain anonymity

Patient and staff names are not disclosed in audit documents or referenced in public discussions or conferences.

Anonymity is maintained during report preparation and dissemination.

Participating staff shall not reveal names.

5.5 Document the audit

A checklist with predefined parameters could be used and findings could be recorded on it.

After the audit, a report shall be prepared highlighting key findings.

5.6 Complete the audit cycle

All ascertained remedial measures are documented and implemented.

Improvements are recorded to complete the audit cycle.

Root-cause analysis is preferably used as the basis.

6. Governance and responsibility

Role	Responsibility
Organisation	Defines audit parameters, identifies participating personnel, conducts the required audits and ensures documentation and remedial measures.
Audit personnel	Identified personnel participate in clinical audits and maintain patient and staff anonymity.
Clinical departments	Each clinical department participates in the required audit cycle according to the scope of services.
Responsible personnel	Ensure findings are reported and remedial measures are documented, implemented and improvements recorded.

7. Quality monitoring

Monitoring area	What is monitored
Clinical audit frequency	One clinical audit per clinical department once in two years as per scope of services
Audit parameters	Objectives, standards, checklist where required, sampling/data-collection guidelines and report preparation
Care coverage	Clinical and nursing care
Personnel	Identified personnel participating in clinical audit
Anonymity	Patient and staff anonymity during report preparation and dissemination
Documentation	Post-audit report highlighting key findings
Remedial measures	Documentation, implementation and recording of improvements

8. Training and staff acknowledgement

Staff participating in clinical audits are familiar with the processes described in this policy.

I have read the Policy on Clinical Audit of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to personnel involved in clinical audits.

The processes for clinical audit are communicated to relevant staff.

10. Abbreviations

Abbreviation	Meaning
QA	Quality Assurance

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
PSQ.5.a	Commitment	Sections 3.1 and 5.1 address clinical audits, the exact

		frequency, optional audit types/topics, and distinction from research projects.
PSQ.5.b	Commitment	Sections 3.2 and 5.2 address predefined parameters, objectives, standards, checklist where required, sampling/data-collection guidelines, report preparation, and clinical/nursing care.
PSQ.5.c	Achievement	Sections 3.3 and 5.3 address identification of participating personnel while preserving optional personnel composition and committee mechanism.
PSQ.5.d	Commitment	Sections 3.4 and 5.4 address anonymity during report preparation and dissemination and non-disclosure of names.
PSQ.5.e	Commitment	Sections 3.5 and 5.5 address documentation, optional checklist use and mandatory post-audit report.
PSQ.5.f	Commitment	Sections 3.6 and 5.6 address documentation, implementation and recording of improvements, with root-cause analysis preferred rather than mandatory.

12. Required Records/Evidence Checklist

Clinical audit planning and frequency

- Clinical audit schedule showing one clinical audit per clinical department once in two years as per the scope of services.
- Audit plan showing the selected clinical audit topic and approach.

Audit parameters

- Documented audit objectives and standards.
- Checklist where required.
- Sampling and data-collection guidelines.

- Documentation showing clinical and nursing care are included.

Personnel and anonymity

- List of personnel identified to participate in clinical audits.
- Audit documents and processes showing patient and staff names are not disclosed during report preparation or dissemination.

Audit documentation

- Post-audit report highlighting key findings.
- Checklist or other audit finding record where the organisation has chosen to use one.

Remedial measures

- Documented ascertained remedial measures.
- Records showing remedial measures were implemented.
- Records showing improvements were recorded to complete the audit cycle.
- Root-cause analysis where it has been used as the basis for remedial measures.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PSQ.5.

Disclaimer

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